



STRATEGY & OPERATIONS

Vytalix Market Strategy

Vytalix company-build documentation · reference
05_MARKET_STRATEGY

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PREPARED FOR
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Contents

Executive view	1
1. United Kingdom — home market	2
2. Africa — six markets scored	3
3. Europe	4
4. Middle East — UAE and Saudi Arabia	5
5. North America	6
6. Other emerging markets — India, Bangladesh, Pakistan, Indonesia	7
7. Market score table	8
8. Entry sequence and verdicts	9
9. Go-to-market plays per market (summary)	10
Priorities / Risks / Next actions	11

Owner: Chief Strategy Officer · **Status:** PROPOSED v1 · **Date:** 7 September 2026 · **Governing file:** /FACTS_BASE.md . Every figure taken from the web carries its URL; everything else is ESTIMATE, ASSUMPTION or PROPOSED. Vytalix has no customers, pilots or partners in any market today (KNOWN).

Executive view

- 1. **The UK is the home market and the credibility market**, scoring highest (67.9/100) on the eleven-criterion model in \$7 because of policy pull (MBRRACE-UK disparities, the national maternity investigation, the digital maternity record standard), funding depth and partnership density — not because it is fast. NHS product sales cycles are 9–18 months; the first UK revenue is services, then an unpaid design partnership, then a paid pilot (month 12–24).
- 2. **Nigeria is the volume and impact market** (61.3 raw; 55.2 feasibility-adjusted) and the only African market where Vytalix has a live conversation (Ekiti, status unknown) — but that state has a funded incumbent (mDoc / MSD for Mothers, VERIFIED). The play is "partner or move state", scored against Ondo, Osun, Oyo, Kwara (Yoruba content reuse) and Kaduna/Kano (burden), with NGO- and donor-funded cohorts preferred over state budget lines until willingness to pay is validated.
- 3. **Kenya is the third market, and the second African one** (58.9 raw; 47.1 adjusted): strongest digital and mobile-money rails, county-level buyers, an active donor ecosystem — and the most instructive incumbent, Jacaranda Health's PROMPTS (2.8m+ women, <\$1/mother), which sets the price expectation Vytalix must design around. Ghana and Rwanda are "prepare, do not enter" until 2028; South Africa and Ethiopia are "do not enter yet".
- 4. **Everything outside the UK-Africa corridor is "do not enter yet"** for the product: Europe (no differentiated need, procurement fragmentation), UAE/Saudi (attractive but relationship- and localisation-heavy, no fit before an evidence base), North America (highest raw score but a crowded, well-funded field — Maven at a \$1.7bn valuation, Babyscripts, Bloomlife, Nuvo — and a naming collision with Vytalix Inc.), and the South Asian/Indonesian markets (huge need, no channel, no capital). Consulting can serve these markets opportunistically; the product cannot before 2029.
- 5. **Entry sequence:** UK services (now) → UK design partner + Nigeria NGO/state-adjacent cohort (months 6–18) → UK paid pilots + Kenya county/NGO programme (months 18–36) → Ghana/Rwanda via a regional partner (2028+) → Gulf/Europe/US only with evidence, a Series A and a partner-led model (2029+). Each step has a stated go/no-go signal in \$8.

1. United Kingdom — home market

1.1 Structure that matters to a maternity product and a health consultancy

Element	What it is (KNOWN unless labelled)
NHS England and DHSC	National policy, funding rules and standards; NHS England is being absorbed into DHSC (announced 2025 — TO VALIDATE current state and timing). National matern programme, Maternity Incentive Scheme (via NHS Resolution), digital standards.
Integrated Care Boards (42 in England)	Commission services for their population; host Local Maternity and Neonatal Systems (LMNS); commission Maternity and Neonatal Voices Partnerships (MNVPs) (NH: England, https://www.england.nhs.uk/long-read/three-year-delivery-plan-for-maternity-and-neonatal-services/). ICBs were asked to cut running costs substantially (TO VALIDATE).
NHS trusts providing maternity	About 120 trusts in England provide maternity services (ESTIMATE; NHS England maternity statistics cover ~120 providers — TO VALIDATE exact count), delivering ~56 in England in 2024 (ONS: 567,708 live births in England, 26,832 in Wales; 594,677 total, https://www.ons.gov.uk/peoplepopulationandcommunity/birthsdeathsandmarriages/livebirths/bulletins/birthsummarytablesenglandandwales/2024refreshedpop Median trust ≈ 4,000–5,000 births/year (ESTIMATE).
Devolved nations	Scotland (14 health boards), Wales (7 health boards), Northern Ireland (5 trusts) with separate digital programmes and procurement.
National maternity investigation (2025–26)	A rapid national investigation into maternity and neonatal care named 14 trusts (BMJ Group, https://bmjgroup.com/maternity-care-14-nhs-trusts-at-centre-of-invest-named/); a national maternity dashboard to flag safety concerns was to reach all maternity services by November 2025 (Digital Health, https://www.digitalhealth.net/2025/06/dhsc-to-roll-out-national-maternity-dashboard-to-address-failings/).
MBRRACE-UK 2025 report (deaths 2021–23)	Maternal death rate 12.82 per 100,000 maternities (down from 13.56 in 2020–22); 643 women's care examined; Black women more than twice as likely to die as White Asian women also at elevated risk; thrombosis the leading direct cause (NHS Providers summary, https://nhsproviders.org/resources/summary-of-the-2025-mbracc-saving-lives-improving-mothers-care-report ; report, https://www.npeu.ox.ac.uk/assets/downloads/mbracc-uk/reports/maternal-report-2025/MBRRACE-UK%20Maternal%20Report%202025%20-%20Main%20ONLINE%20v1.0.pdf). The 2025 deck's "3x" figure must be replaced by the current "more than twice" wording
Three Year Delivery Plan (March 2023) and the Maternity and Neonatal Single Delivery Plan (2025 successor — TO VALIDATE title, scope and dates; BAPM response at https://www.bapm.org/articles/bapm-responds-to-single-delivery-plan)	Four themes: listening to women and families; growing and supporting the workforce; culture of safety, learning and support; standards and structures. Digital com included a refreshed digital maternity record standard and maternity services data set (MSDS) standard by March 2024 (NHS England, https://www.england.nhs.uk/long-read/three-year-delivery-plan-for-maternity-and-neonatal-services/).
Digital maternity records	Three principal maternity EPR suppliers: Clevermed BadgerNet (acquired by System C, 2023, https://www.healthcareittoday.com/2023/02/24/clevermed-and-its-bad-solution-join-the-system-c-family/), K2 Athena, and Euroking (Magentus). System C states BadgerNet is used by "over 200 trusts" across maternity and neonatal (https://www.systemc.com/healthcare/badgernet-maternity/ — figure includes neonatal units; maternity-specific footprint TO VALIDATE). K2 markets Athena as a res safety concerns raised about Euroking (https://www.k2ms.com/euroking-response-1); Dartford and Gravesham went live with K2 in April 2026 (https://www.digitalhealth.net/2026/04/dartford-and-gravesham-goes-live-with-k2-maternity-system/). NHS England's maternity digital capabilities framework sup procurement (https://www.england.nhs.uk/mat-transformation/harnessing-digital-technology/).
Procurement	Provider Selection Regime (clinical services) and the Procurement Act 2023 regime (goods/services); frameworks (G-Cloud, NHS SBS, HealthTrust Europe, CCS); direct under thresholds (TO VALIDATE current values).
Assurance	DTAC; DCB0129 (manufacturer) and DCB0160 (deploying organisation) clinical safety; DSPT; Cyber Essentials Plus; DPIA under UK GDPR; NICE Evidence Standards Framework MHRA if a medical-device intended purpose is claimed.

1.2 UK sub-segments and the play for each

Sub-segment	Size (ESTIMATE)	Buyer	Play	Timing
NHS trusts (maternity)	~120 in England; 5–10 realistically addressable by 2030	Head/Director of Midwifery; Maternity Digital Lead; CNIO; Divisional Director; Clinical Safety Officer	(1) Consulting first: Digital Maternity Readiness Review (C3), Health Inequalities Review (C8). (2) One unpaid design partner (community midwifery team) under DCB0160. (3) Paid pilot £20k–£60k. Land through Health Innovation Networks, LMNS, MNVPs and a clinical advisor.	Services now; design partner month 6–12; paid pilot month 12–24

Sub-segment	Size (ESTIMATE)	Buyer	Play	Timing
LMNS / ICBS	42 ICBS; each hosts an LMNS with an equity and digital lead	LMNS programme director; ICB Director of Nursing; health-inequalities lead	Inequality reviews, MSDS data-quality work, LMNS digital strategies; MaternaLink as a multi-trust community offer later	Consulting month 3–12
Private maternity	Small: a handful of London units (e.g., The Portland, Kensington Wing, Lindo Wing, Chelsea & Westminster private) plus independent midwives (~100–200 practising, ESTIMATE)	Hospital director; consultant obstetricians; independent midwife groups	Per-woman-per-episode licence (£30–£80, ESTIMATE); fast credibility pilots; fewer IG barriers	Month 9–18
Employers and benefits platforms	Large employers with maternity-return programmes; benefits platforms (competition: Peppy, Maven's UK presence — TO VALIDATE)	HR benefits; DEI leads	Non-clinical education and coordination for expectant employees; sell through benefits platforms; £2–£6 PEPM or £150–£300 per pregnancy (ESTIMATE)	Month 12–24
Universities	Midwifery, nursing, public-health and health-informatics programmes; maternal-health research groups (e.g., NPEU Oxford, King's, Manchester — institutions only, no relationships)	Programme leads; principal investigators	E-Learning licences (£3k–£8k/programme, ESTIMATE); research partnerships for evaluation and the v3 AI track (NIHR, UKRI co-applications)	Month 9–24
Charities and community organisations	Maternal-health charities and Black maternal-health advocacy groups	CEOs; programme leads	Co-design and content partnerships (paid), joint grant bids; never "endorsement" claims	Month 3–12

1.3 UK go-to-market plays (PROPOSED)

- Inequality-led consulting entry.** Offer C8 (Health Inequalities & Maternity Outcomes Review) to LMNSs and trusts named in the national investigation or with CQC "requires improvement" maternity ratings. Evidence base: MBRRACE-UK 2025 and the Committee report above. Target: 6 proposals, 2 wins in 12 months (£28k each, ESTIMATE).
- Digital-stack moment.** Trusts replacing Euroking or re-procuring maternity EPRs in 2026–28 need readiness reviews (C3) — position Vytalix as vendor-neutral.
- Design partner, not customer.** One trust, one community team, DCB0160 with the trust's CSO, no fee; produce a service evaluation and a safety case. This is the asset that sells everything later.
- Health Innovation Network route.** Apply to relevant innovation programmes; use them for introductions, not endorsements.
- Content and community.** Publish a short, sourced "State of digital maternal care UK/Nigeria" report (see marketing plan) to earn the inbound conversations that a solo founder cannot cold-call into.

2. Africa — six markets scored

Context (all TO VALIDATE against the primary reports before external use): global maternal deaths in 2023 were roughly 240,000 (IHME, <https://www.healthdata.org/news-events/newsroom/news-releases/global-maternal-deaths-fell-240000-2023-more-100-countries-still>) to 260,000 (WHO/UN MMEIG estimate — the two series differ methodologically; cite one and say which). Nigeria's modelled maternal mortality ratio in 2023 was 993 per 100,000 live births, the highest in Africa (Statista compilation of WHO/World Bank data, <https://www.statista.com/statistics/1122869/maternal-mortality-rate-in-africa-by-country/>; World Bank series at <https://data.worldbank.org/indicator/SH.STA.MMRT?locations=NG>). Kenya was estimated at 530 per 100,000 in 2020 (same Statista source). Nigeria, India, DRC, Ethiopia and Pakistan recorded the highest absolute numbers of maternal deaths (IHME, above).

Country	Need (burden, gaps)	Digital / mobile	Health system & buyers	Incumbents and partners	Regulation & data	Verdict
Nigeria	Highest MMR in Africa (993/100k, 2023); largest absolute burden; wide North–South gradient; ANC and skilled-birth-attendance gaps (NDHS — TO VALIDATE latest).	High mobile penetration, low smartphone share outside cities; SMS/USSD/WhatsApp essential; frequent connectivity gaps (offline-first is a real requirement).	36 states + FCT each buy separately via Ministries of Health and SPHCDA; federal actors (FMoH, NPHCDA, NHIA); state health insurance agencies; strong private HMO sector (Lagos). State budgets are volatile; donors fund most digital maternal programmes.	mDoc Digital Mom (Lagos, Ekiti; MSD for Mothers-funded; ~24,000 women in Ekiti — VERIFIED per FACTS_BASE); Helium Health (EMR, \$42m raised, https://www.crunchbase.com/organization/helium-healthcare ; has MSD for Mothers and Gates funding for maternal projects, https://techpoint.africa/news/helium-health-30m-seriesb/); numerous NGO programmes. Partners: Society for Family Health, Wellbeing Foundation Africa, Nigeria Health Watch (as convenor) — no relationships (KNOWN).	Nigeria Data Protection Act 2023 (NDPC registration for data controllers of major importance — TO VALIDATE thresholds); NHREC ethics for research; local hosting expectations; UK Bribery Act applies to Vytalix staff.	Enter second (months 6–18) via NGO/donor-funded cohort in a state without an incumbent, or as a complement to mDoc. Do not restate the £4.5m Ekiti proposal.
Kenya	MMR ~530/100k (2020); Linda Mama free maternity scheme now under the Social Health Authority (SHA, 2024–25 transition — TO VALIDATE); uneven county capacity.	Best mobile-money and SMS ecosystem in the region (M-Pesa; Africa's Talking gateway); high phone ownership among women.	47 counties buy independently; national SHA; strong private and faith-based provider networks; established digital-health policy and Kenya Health Information System (DHIS2).	Jacaranda Health PROMPTS: 2.8m+ women, 22 counties, "less than \$1 per mother" (https://jacarandahealth.org/program/prompts/ ; https://www.millionlives.co/members/jacarandahealth/); Totohealth (SMS since 2014, https://www.do4africa.org/en/projets/totohealth-2/); Zuri Health; Penda Health and others.	Data Protection Act 2019 (ODPC registration); Digital Health Act 2023; local hosting preferences.	Third market (months 18–36). Do not compete with PROMPTS on SMS education; position on facility-side coordination, handover and referral with a county or NGO partner.

Country	Need (burden, gaps)	Digital / mobile	Health system & buyers	Incumbents and partners	Regulation & data	Verdict
Ghana	MMR materially lower than Nigeria (ESTIMATE ~250–300/100k; TO VALIDATE); good ANC coverage; NHIS covers maternity.	Good mobile coverage; MTN dominant; growing health-tech scene.	Ghana Health Service is centralised (an advantage: one buyer, one standard); NHIA; teaching hospitals.	mPedigree, mTrac-type programmes; Zipline (logistics); fewer maternal-messaging incumbents than Kenya.	Data Protection Act 2012; Ghana Health Service research approvals.	Prepare (2027), enter via a regional partner (2028+).
Rwanda	Smaller population (~14m); MMR improving strongly; government prioritises digital health.	Good connectivity for the region; centralised systems.	Single national buyer (MoH/RBC); fast decisions; small budgets; strong donor presence.	Babyl (Babylon) exited; RapidSMS for maternal tracking is a long-standing government programme (TO VALIDATE current status).	Data protection law 2021; strong government control of health data.	Prepare; enter only as a government-invited pilot (2028+). A good evaluation site, not a revenue market.
South Africa	MMR far lower than Nigeria; inequality by province; strong public-private split.	High smartphone penetration; MomConnect precedent (1.7m women registered by 2017 via 95% of public facilities, https://en.wikipedia.org/wiki/MomConnect).	National Department of Health with provincial delivery; NHI reform uncertainty; large private medical-scheme sector (Discovery et al.).	MomConnect is a national, government-owned programme (Praekelt); private schemes have mature maternity programmes.	POPIA; NHREC; strong local vendor preference.	Do not enter yet. Government channel is occupied; private channel requires local presence. Revisit 2029.
Ethiopia	Very high absolute burden; Health Extension Worker programme; rural connectivity poor.	Low smartphone and internet penetration; Ethio Telecom liberalising (Safaricom Ethiopia).	Highly centralised federal MoH; donor-heavy; foreign-company operating constraints; FX and conflict risks.	Government-led digital health (DHIS2, electronic community health information system).	Restrictive foreign-business rules; data localisation.	Do not enter yet. Consulting through donors only.

2.1 Nigeria state selection (the "move state" scorecard, PROPOSED)

Criterion (weight)	Ekiti	Ondo	Osun	Oyo	Kwara	Kaduna	Kano
Burden and population (25)	4	5	5	7	5	8	10
Incumbent absence (20)	1 (mDoc)	7	7	5	7	4	4
Digital and connectivity readiness (15)	6	6	6	7	6	5	5
Content reuse (Yoruba/Hausa) (10)	9	9	9	9	7	3	3
Partner availability (15)	5	5	5	8	6	8	8
Budget / donor presence (10)	5	4	4	6	5	8	8
Security and operating safety (5)	8	7	8	7	8	4	5
Weighted /10 (ESTIMATE)	4.6	6.1	6.2	6.9	6.1	6.0	6.6

Reading: Oyo (Ibadan: teaching hospital, NGOs, large population) and Kano (burden, donor density; needs Hausa-first content and a strong local partner) lead; Ekiti is bottom because of the incumbent, unless mDoc partnership is agreed. All scores are desk ESTIMATES to be replaced by a two-week field scan (consulting offer C5 delivered to ourselves).

3. Europe

Aspect	Assessment
Need	Low relative to UK/Africa for the v1 use-case; maternal mortality low; strong midwife-led systems (Netherlands, Nordics) with existing digital records. Migrant and minority-ethnic disparities exist (TO VALIDATE country data) but are politically less salient than in the UK.
Regulation	EU MDR (a non-device intended use must be re-argued under EU rules); GDPR; EHDS (European Health Data Space) interoperability obligations rolling in 2027–29 (TO VALIDATE dates).
Buyers	Fragmented: hospital groups, regions, insurers (Germany DiGA for reimbursed apps; Netherlands insurers); language localisation per market.
Competition	National apps and EPR vendors; DiGA-listed pregnancy apps in Germany (TO VALIDATE).
Verdict	Do not enter yet (product). Ireland is the only candidate (English-language, HSE national maternity strategy, single buyer) for 2029+. Consulting: serve EU health-tech companies entering the NHS (C4) from day one.

4. Middle East — UAE and Saudi Arabia

Aspect	UAE	Saudi Arabia
Need	Low mortality; high private-sector share; interest in patient experience and premium maternity.	Vision 2030 health transformation; large national programmes (Seha Virtual Hospital, national EHR); rising interest in women's health.
Buyers	DoH Abu Dhabi, DHA Dubai, private groups (e.g., Mediclinic, NMC); requires local licensing and often a local partner/free-zone entity.	MoH clusters, NUPCO procurement; large tenders; long relationship-building; Saudisation.
Digital	Excellent; Malaffi/NABIDH exchanges.	Excellent; national platforms.
Competition	Global vendors; premium apps.	Global vendors with local partners.
Verdict	Do not enter yet. Revisit after UK evidence and a Series A, with a distribution partner; consulting to Gulf providers exploring African expansion is a plausible opportunistic offer.	Do not enter yet.

5. North America

Aspect	USA	Canada
Attractiveness	Largest addressable market; deep capital; employer and payer buyers; maternal mortality and Black maternal disparities are national issues.	Provincial single payers; slow procurement; smaller scale.
Competition	Densest in the world for this category: Maven Clinic (\$125m Series F, \$1.7bn valuation, >\$425m raised, https://www.cnbc.com/2024/10/08/womens-health-startup-maven-clinic-raises-at-1point7-billion-valuation.html); Babyscripts (\$37m raised, https://tracxn.com/d/companies/babyscripts/_kWZWDhoM9WH_e-3YqrP_Az16bGgqu9zEXuqNu7s-ISs); Bloomlife (\$40.8m, last \$14m in June 2025, https://pitchbook.com/profiles/company/108812-71); Nuvo INVU (FDA-cleared remote monitoring, partnered with Babyscripts, https://babyscripts.com/babyscripts-partners-with-nuvo-remote-pregnancy-care-solution); Ovia, Pomelo, Oula and others.	Smaller vendor set; provincial pilots.
Regulation	FDA (SaMD), HIPAA, state laws; naming collision with Vytalix Inc. (Texas) and Vitalize Health (New Jersey) — a trademark problem before a market problem.	PIPEDA and provincial laws.
Verdict	Do not enter (product) before 2029 and a rename or coexistence agreement. Use the US only as an investor and benchmark market.	Do not enter yet.

6. Other emerging markets — India, Bangladesh, Pakistan, Indonesia

Market	Need	Channel reality	Competition / incumbents	Verdict
India	Among the highest absolute maternal-death counts (IHME, above); state-level buying; Ayushman Bharat Digital Mission creates interoperability rails.	28 states; Hindi + 20 languages; ASHA community workers; very price-sensitive; strong local start-up ecosystem.	Government platforms (RCH portal, Kilikari SMS programme), ARMMAN (mMitra — large NGO programme), many start-ups.	Do not enter yet. Highest raw score among emerging markets (59.0) but the lowest feasibility for a UK pre-seed company; revisit via a licensing partner only.
Bangladesh	High need; strong NGO sector (BRAC); mobile health precedents (Aponjon).	NGO-led; donor-driven; Bangla content.	Aponjon (MAMA programme legacy), BRAC digital programmes.	Do not enter yet.
Pakistan	Very high absolute burden; Lady Health Worker programme.	Provincial; security and FX risk; Urdu/regional languages.	Sehat Kahani, various donor pilots.	Do not enter yet.
Indonesia	Large population; archipelago logistics; national SATUSEHAT platform.	Bahasa; regulatory localisation; strong local players (Halodoc, Alodokter).	Local telehealth incumbents.	Do not enter yet.

Rule for all four: Vytalix E-Learning may sell into these markets from day one (self-serve, regional pricing); Vytalix Consulting may accept donor-funded work; the product does not enter without a licensing partner and dedicated capital.

7. Market score table

Method: eleven criteria scored 0–10 (10 = most favourable to Vytalix; "customer acquisition difficulty" is inverted so that 10 = easiest); weights sum to 100; raw score = weighted sum/10. A **feasibility gate (0–1)** then discounts for what Vytalix can actually reach in 2026–28 given a solo founder, GBP-denominated pre-seed capital, existing conversations and regulatory cost. All scores are desk ESTIMATES; re-score quarterly and after every market visit.

Criterion	Weight														
Market size	10														
Need	12														
Regulation (ease and clarity)	8														
Competition (room for a new entrant)	8														
Digital infrastructure	8														
Healthcare infrastructure	7														
Government opportunity	9														
Funding (grants, donors, investors reachable)	10														
Partnership potential	9														
Customer acquisition (ease)	9														
Revenue potential	10														
Market	Size	Need	Reg.	Comp.	Digital	Health infra	Govt opp.	Funding	Partners	Acquisition ease	Revenue	Raw /100	Feasibility gate	Adjusted	
UK	6	7	6	5	9	9	6	8	8	4	7	67.9	1.0	67.9	

Market	Size	Need	Reg.	Comp.	Digital	Health infra	Govt opp.	Funding	Partners	Acquisition ease	Revenue	Raw /100	Feasibility gate	Adjusted
Nigeria	8	10	5	5	5	3	7	6	7	4	5	61.3	0.9	55.2
India	10	8	5	3	7	4	5	7	6	3	5	59.0	0.3	17.7
Kenya	5	8	6	4	7	5	6	7	7	5	4	58.9	0.8	47.1
USA	10	6	4	2	9	7	2	9	4	2	8	58.3	0.2	11.7
Ghana	4	7	6	6	6	5	6	5	6	5	4	54.6	0.7	38.2
Rwanda	2	6	8	6	7	5	8	5	6	6	2	54.5	0.6	32.7
Europe (IE/NL/Nordics proxy)	6	4	5	4	9	9	4	6	5	3	5	53.3	0.5	26.7
Saudi Arabia	5	4	5	5	8	7	6	6	3	3	6	51.9	0.3	15.6
UAE	3	3	6	4	9	8	5	6	4	4	6	51.1	0.4	20.4
South Africa	5	6	6	3	7	6	4	5	5	4	5	50.9	0.5	25.5
Indonesia	7	7	5	5	6	4	5	5	4	3	4	50.8	0.2	10.2
Bangladesh	6	8	5	5	5	3	5	4	6	4	3	50.2	0.3	15.1
Ethiopia	7	9	4	6	3	3	5	5	5	3	3	50.0	0.4	20.0
Canada	4	4	5	5	9	8	4	5	4	3	5	49.5	0.3	14.9
Pakistan	7	9	4	5	4	3	4	4	5	3	3	48.1	0.2	9.6

Sensitivity: the UK ranks first under any weighting that gives funding and partnerships at least 15 points combined; Nigeria overtakes the UK only if "need" is weighted above 25 and feasibility is ignored; Kenya overtakes Nigeria if the Nigeria feasibility gate falls below 0.75 (i.e., if the Ekiti relationship proves non-transferable and no alternative state partner is found within two quarters).

8. Entry sequence and verdicts

Wave	Window	Market and mode	Go signal (must be true to start)	No-go / exit signal
0	Now–month 6	UK — services only (Advisory, Consulting); E-Learning global self-serve from month 9	Incorporated; name cleared; PI insurance	<1 signed engagement by month 3 (01_BUSINESS_MODEL . md §8)
1	Months 6–18	UK — MaternaLink design partner (one trust, unpaid) · Nigeria — NGO/donor-funded cohort in a scored state, or a complementarity agreement with mDoc in Ekiti	IP assigned; prototype audited; CSO appointed; DPIA done; a clinical adviser; a Nigerian implementing partner with an existing cohort	No UK trust interest after 10 approaches → product focuses Africa/private; no Nigerian partner within two quarters → pause Nigeria, advance Kenya
2	Months 18–36	UK — paid pilots (2–3 trusts, private maternity) · Kenya — county or NGO programme positioned on facility coordination/referral alongside PROMPTS-type messaging	UK service-evaluation evidence and safety case; Kenya partner (NGO or county) and ODPC registration; SMS cost model validated	Pilot NPS/usage below thresholds in 04_MATERNALINK_PRODUCT_STRATEGY . md §24
3	2028+	Ghana, Rwanda via a regional implementing partner; Nigeria second state	Positive evaluation from wave 1–2; Series A or equivalent grant portfolio; local hosting option	Absent evidence → stay at two African markets
4	2029+	Ireland, Gulf, North America — partner-led or licensing only; South Africa, Ethiopia, South Asia, Indonesia — licensing only	Trademark position resolved (US); localisation budget; a channel partner per market	Never enter direct

"Do not enter yet" verdicts (product), with the single fact that would change each: Europe (a national single-buyer tender in Ireland); UAE/Saudi (a distribution partner willing to fund localisation); USA/Canada (a coexistence agreement or rename, plus Series A); South Africa (a private-scheme partner); Ethiopia (foreign-operating rules and connectivity); India/Bangladesh/Pakistan/Indonesia (a licensing partner with an existing cohort and local capital).

9. Go-to-market plays per market (summary)

Market	Lead pillar	Wedge offer	Channel	Price anchor (ESTIMATE)	12-month target
UK	Consulting → Health Solutions	C8 inequalities review; C3 readiness review; unpaid design partnership	Health Innovation Networks, LMNS, MNVPs, clinical advisor, sourced report	£14k–£28k per sprint; pilot £20k–£60k (later)	2 sprints; 1 design partner
Nigeria	Advisory → Health Solutions	Re-scoped, partner-aware pilot (≤ £0.5m/yr programme envelope; platform £6–£40/woman/yr)	Implementing NGO; state MoH/SPHCDA; donor programme officers; diaspora clinician networks	Platform £6–£40/woman/yr + SMS pass-through	1 NGO/donor cohort scoped; Ekiti status resolved; mDoc conversation held
Kenya	Consulting → Health Solutions	County/NGO facility-coordination and referral module	Africa's Talking-type gateway; NGO partner; county health executive	£3–£12/enrolled pregnancy (insurer/HMO) or programme licence £8k–£60k/yr	2 discovery visits; 1 partner LOI
Ghana / Rwanda	Consulting	Market-entry scans for third parties (C5) that double as Vytalix's own intelligence	Donor programmes; regional health-tech networks	C5 £20k	1 paid scan
Europe / Gulf / North America	Consulting (C4 UK Market-Entry Playbook for inbound vendors)	Serve vendors entering the NHS	Health-tech accelerators; investors	C4 £22k	1–2 engagements

Market	Lead pillar	Wedge offer	Channel	Price anchor (ESTIMATE)	12-month target
South Asia / Indonesia	E-Learning	Regionally-priced courses	Self-serve; professional associations	£10–£40/course	100 learners

Priorities / Risks / Next actions

Priorities

1. UK: 15 discovery interviews (midwives, digital midwives, LMNS leads, private units) by day 60; two consulting proposals to LMNS/trusts by day 90; one design-partner conversation opened through a clinical advisor.
2. Nigeria: resolve the Ekiti status in one call; one exploratory conversation with mDoc on complementarity; commission (internally) the state scorecard field scan for Oyo and Kano.
3. Kenya: desk research into county priorities and SHA maternity coverage; two partner conversations (an NGO and a messaging gateway) by month 6.
4. Keep every other market to consulting and e-learning only; record inbound interest in the CRM, do not chase.
5. Re-run the score table at month 6 with field data and the FCDO/donor funding calendar.

Risks

- UK reorganisation (NHS England into DHSC; ICB cost cuts) delays commissioning decisions → sell to trusts and LMNSs, not to bodies being restructured.
- Nigeria: the Ekiti relationship is not transferable to Vytalix, or the state has moved on → the state scorecard is the mitigation; NGO cohorts do not depend on any one ministry.
- Kenya: PROMPTS' <\$1/mother benchmark makes any per-woman price above a few dollars look unrealistic for messaging alone → position on coordination and facility workflow, price to the programme not the message.
- Naming collision blocks any North American ambition → resolve trademark strategy before an investor asks about the US.
- Founder capacity: each additional market costs ~20 days/year of unbilled time → no more than two active product markets before 2028.

Next actions

- Week 1–2: build the UK interview list (40 names, roles not people, from public sources) in the CRM; draft the interview guide.
- Week 2: Ekiti status call; mDoc outreach email (see partnership strategy for wording rules — no claimed relationship).
- Week 3–6: state scorecard desk phase; Kenya partner desk research; verify every web-cited figure above against the primary source and record the check date.
- Month 3: first re-score; decide whether Kenya moves ahead of a second Nigerian state.